

Tilted uterus

» What is it?

The uterus is normally in a straight, vertical position. A tilted uterus is one that tips backward toward the spine. It's also known as a retroverted uterus. Symptoms of tilted uterus usually include pain during sexual intercourse and pain during menstruation.

» What does this condition mean for my pregnancy? Will my uterus stay tilted after I've given birth?

The main implication for your pregnancy is that your baby may be hard to spot during an abdominal ultrasound. Some women with a tilted uterus have to have a vaginal

ultrasound in order for the sonographer to be able to see the baby from all angles. This is simply because the baby is farther away from the front of the abdomen. Other than that, complications arising directly from your uterine anatomy are extremely rare—once you have completed 12 weeks of pregnancy, your uterus will expand out of your pelvis anyway and as it grows will fill the space in your abdomen. After birth, your uterus may or may not move back into its original position—it partly depends on the strength of the muscles and ligaments around your uterus. Whatever happens, the position of your uterus after you've had your baby is unlikely to affect your chances of conceiving again in the future, so try not to worry about it.

Endometriosis

» What is it?

In this condition, pieces of tissue that are normally part of the uterine lining (also called the endometrium) are found in the abdomen and pelvis, including on the ovaries, in the bowel, and around the bladder. The endometrium is the part of the uterus that sheds during your periods. This condition can result in heavy, painful menstruation and infertility, but the cause of it is not totally understood.

» I was diagnosed with endometriosis a couple of years ago. Could it affect my pregnancy?

Once pregnant, endometriosis is unlikely to affect your pregnancy, your baby, or increase any risk factors. (The main

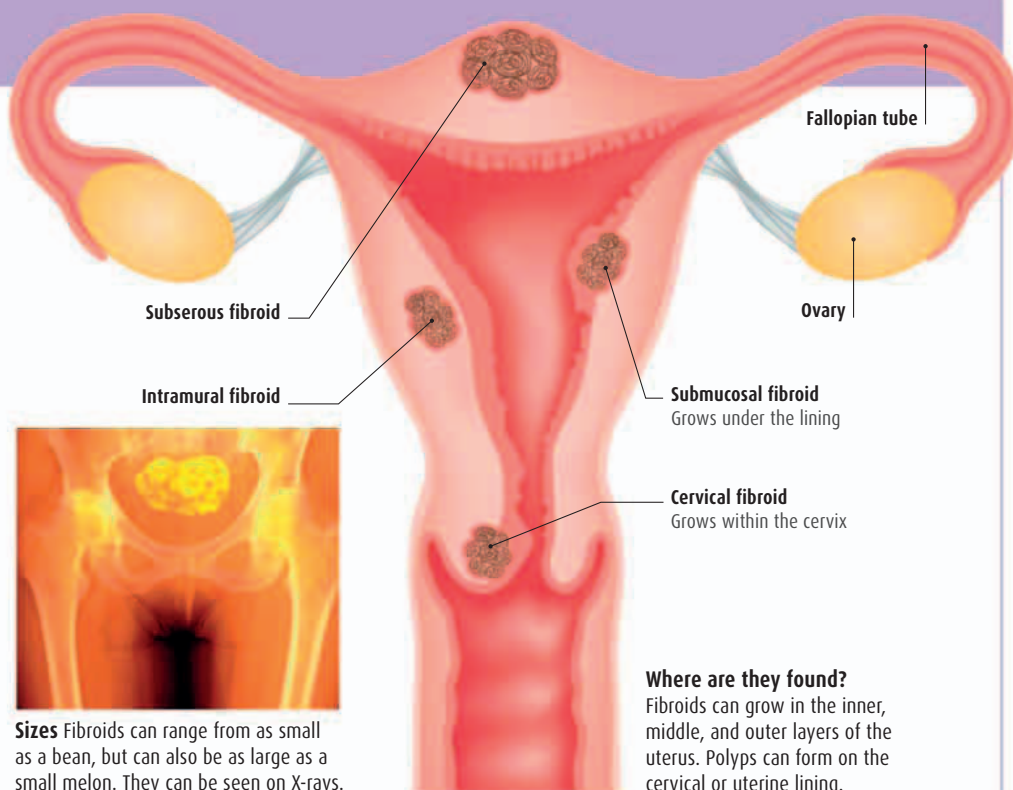
Fibroids

» What are fibroids?

Fibroids are noncancerous growths in the uterus, affecting a quarter of all women at some point in their lives. The most common form grow within the muscular wall of the uterus (intramural fibroids), although they may also appear outside the muscle wall, growing into the pelvis (subserous fibroids); dangling from the lining of the uterus (pedunculated fibroids); embedded in the inner lining of the uterus (submucosal fibroids); or on the neck of your cervix (cervical fibroids). They may be small or large, and their exact cause is unknown, although excess estrogen makes them bigger. (This is why women who are overweight are more likely to have fibroids, because fat cells produce estrogen.)

» How will I know if I have fibroids and what effects will they have on my body and baby?

Fibroids may cause painful and/or heavy periods, pain during intercourse, abdominal bloating, or frequent urination (if the fibroid presses on the bladder). If fibroids are large or numerous, they can affect fertility because they make it harder for the embryo to implant in the uterine lining, and they may also make it more likely for you to miscarry if you do get pregnant. If you have had fibroid treatment pre-pregnancy, this can affect your delivery options once pregnant, so talk to your doctor about this. Many fibroids are asymptomatic—in other words, a large number of women don't even discover they have them until they have a routine pregnancy ultrasound, until the fibroids cause spotting during pregnancy, or until their doctor presses on their uterus to feel the position of the baby. Some fibroids restrict space in your uterus so that your baby doesn't have the space to grow properly. However, this is very rare. Also, as fibroids grow under the influence of pregnancy hormones, they can start to degenerate, usually in the



Sizes Fibroids can range from as small as a bean, but can also be as large as a small melon. They can be seen on X-rays.

Where are they found?

Fibroids can grow in the inner, middle, and outer layers of the uterus. Polyps can form on the cervical or uterine lining.

second trimester, and this can cause pain and, rarely, premature labor.

» My fibroids are large and my doctor has said that I may have to have a C-section. Why?

There are several reasons why fibroids might mean that C-section is the safest way to deliver your baby. In pregnancy, a large fibroid may push your baby into an awkward position for delivery (called "malpresentation"), including the breech position and very occasionally presenting with a shoulder first. Cervical fibroids may block the entrance to your cervix, making it difficult for your baby to pass into the birth canal. If fibroids cause your placenta to come away slightly from your uterine lining (partial abruption; see opposite), your baby will

continue to be well-nourished, but a preterm C-section, as soon as your baby is big enough to survive outside the uterus, will remove the risks to the baby if the placenta comes away completely. Finally, fibroids also put you at slightly increased risk of postpartum hemorrhage (excessive bleeding after the birth) and preterm delivery. Your doctor has a duty to ensure you are fully aware of any potential complications, but it's important to remember that fibroids only rarely cause any problems and all the situations given here are exceptional. Fibroids can be removed after pregnancy and if you do not want more children you can have a hysterectomy which means there is no risk of them reoccurring.